

Non-Acute Rehabilitation Pathways Service

Operational Guidelines

A variation of the Non-Acute Rehabilitation Service

This document is a living document and will be reviewed and updated as necessary.

How to read this guide

The following documents may be helpful when reading this operational guide.

- [ACC's Standard Terms and Conditions](#); and
- [Non-Acute Rehabilitation Pathways Service Specification](#)

This guide supports the Non-Acute Rehabilitation Pathways (NARP) Service schedule. If there are any inconsistencies between these operational guidelines and the service specification, the contract takes precedence.

ACC will work collaboratively with Suppliers to support the intended outcomes and to improve the operation of the service. We will keep this guide up to date with any improvements or refinements made.

About the Contract

Non-Acute Rehabilitation Pathways (NARP) is a Casemix funded model of services.

Non-Acute Rehabilitation Pathway (NARP) service funding is case-weighted packages.

These Operational Guidelines are for the NARP service schedule only.

Note: Where words are capitalised within a sentence the term has been defined in the Glossary.

Purpose

(Refer to Part B, Clause 1 of Service Schedule)

This guide provides information for hospital service managers and clinicians. It elaborates on clauses in the NARP Service Schedule.

The NARP service spans the patient journey, including inpatient and community services, transitional care and rehabilitation admission avoidance pathways. It supports best practice rehabilitation by acknowledging that rehabilitation in an inpatient setting is not always the best option for clients and by providing the opportunity for delivery of innovative rehabilitation within the community setting.

The NARP service is available to all injured Clients, without age restriction, where they meet the criteria to receive ACC funding for hospital provided rehabilitation and care. Flexibility of service delivery is one of the strengths of the NARP service model. NARP services enable hospitals to manage the Client's rehabilitation across the hospital-community continuum, so long as the Client is actively participating in rehabilitation. The delivery of NARP services is to be guided by the interdisciplinary team with the rehabilitation plan established by a registered health professional.

In the community setting, the NARP framework enables support workers/healthcare assistants/allied health assistants to provide rehabilitation for NARP clients under the oversight of registered health professionals.

Under the NARP service contract, hospitals are responsible for the continuum of care across inpatient and community settings.

The purpose of the NARP Service is to return ACC Clients of all ages to their pre-injury function or maximum independence by providing inpatient and community-based rehabilitation and support.

Service funding and pricing

(Refer to Part A, Clause 4 of Service Schedule)

Entitlement to ACC Funding

The Supplier will:

- Liaise closely with their Emergency Department and Acute wards to identify Clients that would require inpatient, community rehabilitation or transitional care when the medical or surgical treatment is complete. The Client is ready (well enough) to participate, whether that be in the inpatient, community or transitional care setting.
- Determine if the eligibility criteria have been met.
- Ensure the Client has ACC claim cover before starting ACC funded services.
- Ensure that completion of the interRAI assessments happen only with staff trained and qualified to do so.
- Ensure that the eligibility assessment is reviewed by a Registrar or Consultant who is trained to assess the Client for admission to this Service.

Responsibilities

This section summarises the responsibilities ACC and the supplier have within this funding model.

The **Supplier** is responsible to ACC for:

- Accurate profiling of clients
- Completing the data collection accurately
- Participating in eligibility reviews when required
- Submitting admission and discharge data every month
- Participating in the governance of the service, when invited
- Maintaining a positive relationship based on;
 - Mutual respect
 - Meet all cultural safety practices and guidance, and where applicable, comply with ACC's cultural Safety expectations. See [link](#)
 - Open and transparent communication
- Ensuring they operate a robust business model focused on improving outcomes for clients and reducing operating costs by continually refining and improving services

ACC is responsible to the Supplier for:

- Analysing the data submitted in a transparent and open way
- Undertaking all required analysis when planned
- Periodically reviewing the case-weights
- Participating in the governance of the service which includes;

- Collaboration
- Maintaining a positive relationship based on;
 - Mutual respect
 - Open and transparent communication
 - Being open to the trialling of new services
 - Paying invoices in a timely fashion

Case-weight funding

The case-weight funding aligns with the case-weight coding within the service schedule (Part A).

The NARP casemix model provides clinically meaningful descriptions of individuals and enables allocation into clinically similar groups. This provides a framework to enable flexibility of service delivery while minimising unwanted variation of administrative processes. Casemix models are guided by two principles; clinically meaningful assessment profiles and case-weights based on rehabilitation needs.

Casemix profiles are established to describe client complexity and for allocation into clinically similar groups.

Case-weight rates are the payment for claims accessing a pathway after the profile tool is administered by a health professional. Each client will have different rehabilitation needs. The case-weight rate is based on the average level of resource required across claims with that level of complexity.

The Casemix model is designed to accommodate for these individual differences by addressing the wider group as a whole.

Management of the Casemix Model

The management of the NARP Casemix funding model and contract is a shared responsibility between the Te Whatu Ora District Providers with the NARP contact and ACC.

It will include the following functions:

- Funding reviews
- Data monitoring
- Governance
- Annual Forums

Te Whatu Ora District Providers and ACC will work in collaboration to monitor the service and provide advice on refinements to the casemix model.

Data monitoring will review the case-weight pricing and distribution trends compared to the National averages. ACC Portfolio team will discuss any price variance with Te Whatu Ora District Providers when it is required.

An annual quality forum will be held with all Te Whatu Ora District Providers and ACC to report on NARP Rehabilitation data and outcomes and share quality improvement initiatives.

ACC will liaise with Te Whatu Ora District Providers to establish the most appropriate way in which for these discussions to occur moving forward.

NARP Forums will change as all hospitals transition to the NARP contract and Te Whatu Ora will be notified of these once finalised.

Monthly Invoicing

The supplier will invoice ACC at least monthly on discharge from each NARP pathway. This will be sent to ACC using Send Invoice or API portal.

Admission and discharge dates from the pathways are required for billing.

Table 1: NARP Invoicing

Invoice for	Method	Frequency	Due Date
Client Case-weight coding	Send Invoice or API	On discharge	at least monthly
Treatment Injury service item code	Send Invoice or API	As required	N/A
Exceptional Travel service item code	Send Invoice or API	As required	N/A
Inpatient exceptional length of stay	Send Invoice or API	As required	N/A

Note: Treatment Injury and Exceptional Length of Stay require prior approval and need to be invoiced against a purchase order

Service Location

(Refer to Part A, Clause 2 of Service Schedule)

The service can be delivered in the following locations identified in Table . The intent is that where a Client receives their rehabilitation, is determined by the individual needs of the Client, in a safe environment where they can **actively participate** in rehabilitation.

Table 2: NARP Service Location

	Supplier Facility/ satellite hospital	Client's home or family residence	Residential Facility /Satellite hospital
Rehabilitation Admission Avoidance		✓	✓
Transitional Care	✓		✓
Inpatient Rehabilitation	✓		✓
Community Rehabilitation		✓	✓

The Supplier must provide Community Rehabilitation across 90% of their geographical region.

Service Outcomes

(Refer to Part B, Clause 2 of Service Schedule)

ACC will measure the success of their Service based on the following objectives:

- The reduction in Length of Stay within an inpatient pathway
- The Client ultimately returns to their pre-injury place of residence wherever possible; and
- The Client is satisfied with the services.

Service Eligibility

(Refer to Part B, Clause 5 of Service Schedule)

Eligibility Criteria

Following an acute presentation, the Client is assessed by a clinician for eligibility for ACC NARP services using the NARP Eligibility Tool (Figure 1). A client is eligible for NARP services if they:

Have sustained an injury that has been accepted for cover by ACC; and

Require rehabilitation that would traditionally require hospital inpatient rehabilitation primarily for that covered injury;

- While the Client may have accepted cover for an injury, that injury must be the predominant reason for the rehabilitation.

Is ready for rehabilitation;

- The Client's overall health and function is good enough that the Client can participate.
- Any medical treatment does not interfere with the Client's active participation.

Is capable and willing to actively participate in rehabilitation;

The Client is:

- **Capable** – Their overall physical and cognitive function is such that they can and do take part.
- **Willing** - They want to participate. They demonstrate motivation to participate.
- **Actively participate** – They're not just receiving rehabilitation therapy passively. They're consciously taking part in rehabilitation activities and therapies
 - A Client can actively participate while non weight bearing on the affected limb such as using a walking frame and supporting their weight on the uninjured limb.
 - Most Clients will not be on bedrest but there can be some situations where a Client is actively participating while on bedrest.
 - Clients who are unwell, can't or are unwilling to take direction, aren't in a rehabilitation programme as defined in this guide, so are not funded under this service.

Note: Unwillingness would not be the sole reason for ACC not funding as Providers are experienced at successfully encouraging Clients to actively engage.

Have achievable rehabilitation goals that will improve their functional independence.

- The main goal of the rehabilitation is to return the Client to their pre-injury function.
- An achievable rehabilitation goal is where it's reasonable to expect (based on the Client's overall presentation) that the Client's physical and cognitive function and their participation in the activities of daily living can be improved with the delivery of rehabilitation therapies. The goals are to be client centred and developed in discussion with the client.

Determining who pays for rehabilitation is complex and requires further consideration over "Does the Client need rehabilitation?" The clinician is required to determine if the rehabilitation need is primarily injury related or health related by completing a comprehensive assessment. As there are no shared funding arrangements for NARP, the test is that '***in the main***' the rehabilitation is needed for a covered injury and NOT a health or age-related need.

NARP eligibility criteria is not met

If the primary cause of the need is determined to be the health condition/s the rehabilitation and support services are covered by Ministry of Health funding. For example: A client fainted at home which was recorded as a fall. The client's fainting episode was health related and no physical injury requiring rehabilitation was diagnosed. Therefore, this admission should not be under ACC funding and the client's needs should be met through Ministry of Health funding.

If incorrect funding decisions are made, ACC can and will seek to recover the funding on a case-by-case basis. Should there be ongoing entitlement errors, ACC will work with the Supplier to determine an appropriate solution.

Note: If clarification is required from ACC please contact ACC via claims@acc.co.nz or phone 0800 101 996.

Clinicians should use the diagram below to assess eligibility for ACC NARP services.

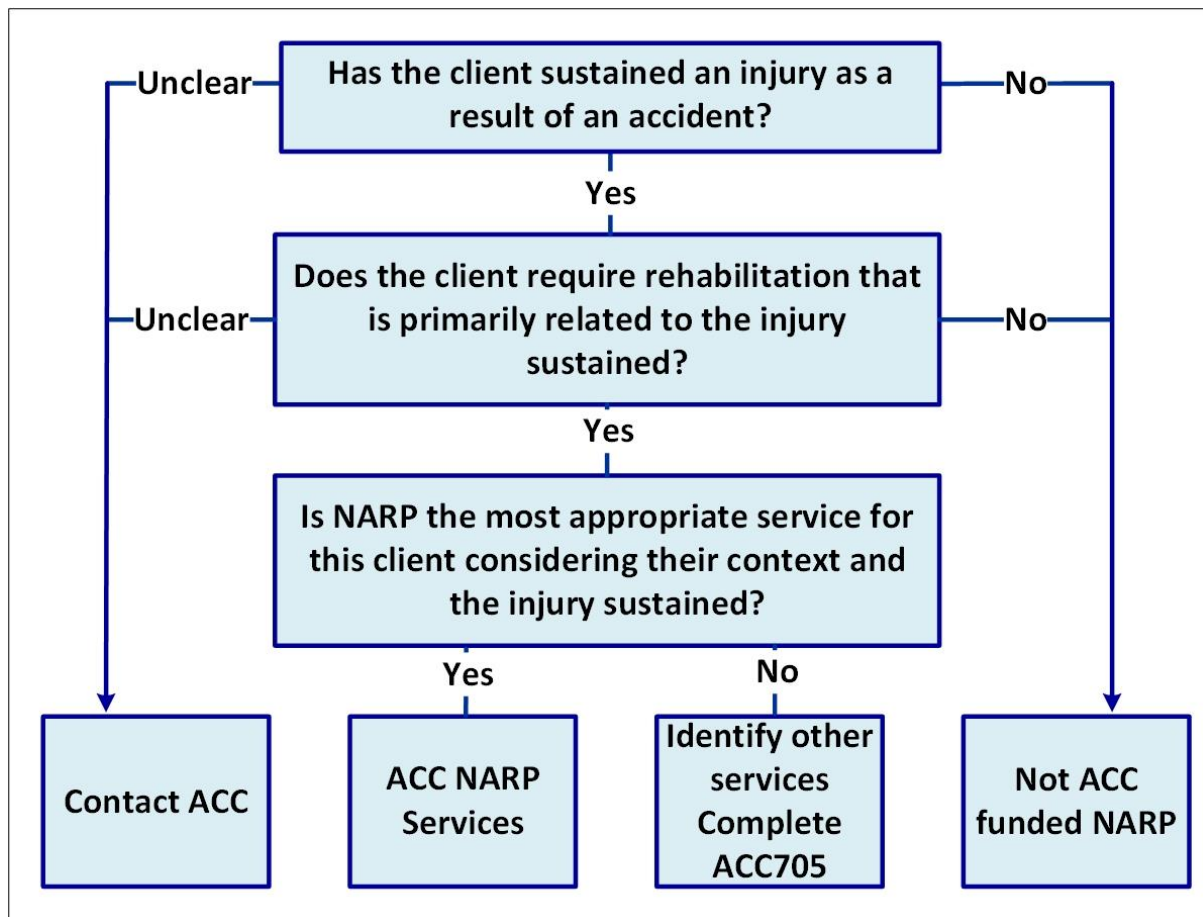


Figure 1: NARP Eligibility Tool

Referral Process

Clients may be admitted or readmitted to the Service from:

- General Practice,
- Emergency Department,
- Acute wards, or
- Rehabilitation ward

Note: Where a client is admitted into NARP services via a GP referral, the hospital is responsible for determining whether the client meets entry criteria to NARP services and to complete profiling for NARP pathway allocation.

Notification of NARP pathway to ACC

ACC needs visibility over claims to see what services a client is accessing, to mitigate duplication of services and enable clients to access additional services, as required, on discharge from NARP services.

On admission into NARP pathways, hospitals will complete the NARP Master Spreadsheet or alternative data capture with patient details.

Once the admission date is entered into the Master Spreadsheet, an email will auto populate a notification that must be sent to ACC. Simply check the details and send.

When hospitals are not using the Master Spreadsheet, they will need to email ACC with the admission details. The email address to be used is recoveryadmin@acc.co.nz This notification will be filed on the client's ACC claim.

Example of the auto populated email from Master Spreadsheet

 Send	To	Recovery Administration Team
	Cc	
Subject		New NARP Inpatient service - NO ACTION REQUIRED

Hi there,

We have just admitted Mickey Mouse for Inpatient Rehabilitation, their details are:

NHI Number: 33

First Name: Mickey

Surname: Mouse

Claim Number: 99999

Inpatient Start date: 25/09/2023

Group / Class: 3A

Major injury type: Fracture_lumbar_spine_pelvis_shoulder_and_upper_arm

ICD10 Code: S32 Fracture of lumbar spine and pelvis

Regards,

Lakes District Health Board, Vendor number: 0, Provider number: T96203

Service Requirements

(Refer to Part B, Clause 6 of Service Schedule)

Minimum Service Requirements for all NARP Rehabilitation Pathways

The Supplier will need to be able to provide a seven-day service for all Rehabilitation Pathways, as required to meet client need, which includes but is not limited to;

- A pre-transfer visit, or consultation with acute team/GP to identify whether a Client is suitable for rehabilitation
- Rehabilitation Specialist oversight (inpatient) and input through active participation in weekly case conferences
- Rehabilitation Specialist oversight (community) should be available, as required. There must be appropriate access to a medical practitioner throughout NARP services and these services must be fully funded within the allocated case-weight.
- Consultations with specialists for any co-morbidities or other issues, as appropriate
- Rehabilitation from an interdisciplinary team, who have a shared philosophy and functional outcome goals

- All investigations, such as laboratory tests, X-rays (basic and specialised, i.e. CT, MRI) and angiography.
- Nursing cares, including pressure injury prevention
- Personal care services
- Medical consumable supplies and pharmaceuticals
- Any equipment required by the Client to meet their needs and to achieve a suitable rehabilitation outcome (this is funded outside of the NARP case-weights). Equipment requests must be made in accordance with ACC's MRES Operational Guidelines. These are available on the ACC website (www.acc.co.nz).
- Transport for tests, assessments, and rehabilitation while in inpatient care.
- Education for caregivers and/or family to enable them to care for the Client after discharge where required
- Planned safe admission and discharge from and to each NARP rehabilitation pathway service.
- Planned safe discharge, as outlined in Service Exit Process
- Access to information and education about injury prevention
- Referral into community or in-home strength and balance programmes, as required on discharge, to prevent injury
- Interpreter and advocacy services
- Māori Health/Liaison Worker and Pacific Island Health/Liaison worker
- Audiology, Optometry and Podiatry relevant to the Client's injuries
- Hotel services including hydration and nutrition (includes dietetics and nutritional advice) within an inpatient setting.
- All incidental services which are reasonable, and that Suppliers of similar services would provide to meet the physiological, cultural, spiritual and social needs of Clients while in the care of the Supplier.

Rehabilitation Principles

The Supplier will provide high quality Rehabilitation and care services. (Part B clause 6 of Service Schedule)

Little 'r' versus Big 'R' rehabilitation

Rehabilitation can be described as big 'R' rehabilitation and little 'r' rehabilitation.

There must be differentiation between big 'R' rehabilitation and little 'r' rehabilitation.

- Little 'r' describes rehabilitation where there is a focus on preventing complications. The client is assisted and encouraged to move, as able, to prevent the consequences of prolonged lack of movement. The need for rehabilitation intervention is usually short term and may be particularly needed after surgery. The client usually recovers function quickly and generally does not need intensive or ongoing rehabilitation.
- Big 'R' describes rehabilitation where there has been significant loss of function. There is a need for therapist guided intervention which gradually progresses the client's functional abilities over time and may require the client to learn to complete a functional activity in a new way, either temporarily or permanently.

When a Client is well enough to **actively participate** in Rehabilitation (big R), ACC purchases that rehabilitation directly through the NARP services.

Within the casemix service delivery, hospitals can use the flexibility of the funding structure to determine how to provide the right care response, in the right location, for individual clients.

Where appropriate in the inpatient and community setting, health care/therapy assistants can deliver aspects of rehabilitation with oversight, input and clinical decision making from registered health professionals. The input of different healthcare workers will vary according to client complexity and need. Hospitals are encouraged to provide innovative and efficacious solutions to deliver interdisciplinary rehabilitation that best align to individual client needs.

Rehabilitation Plan

The rehabilitation plan will be developed with the Client, their family/whānau and primary carer. The plan will include achievable Client centred goals and the therapeutic strategies to attain those goals. The plan will be available to the Client on an ongoing basis in the Client's personal rehabilitation folder.

All Providers and support workers will have easy access to the rehabilitation plan, that describes what the Client will be doing and how Providers are to support the Client. It will include how many hours per day the Client should be participating in rehabilitation and what therapy will be delivered. The plan will be provided to ACC on request.

Pathways Management

(Refer to Part B, Clause 6 of Service Schedule)

The Supplier will manage the coordination and delivery of the Client's rehabilitation throughout the NARP service pathways, from when the Client enters, to when the Client exits the service.

A client's pathway may include various stages of the NARP rehabilitation service.

- Clients can access one or more of the NARP rehabilitation service stages per claim
- There is only one case-weight at the beginning of each of the NARP rehabilitation stages. Moving between the NARP rehabilitation services will only occur once, unless a readmission into the inpatient rehabilitation pathway is required, or the client requires transitional care.
 - Where a client is readmitted into inpatient rehabilitation, the inpatient profiling tool may need to be utilised a second time.
 - Following discharge from hospital, readmission into inpatient rehabilitation less than seven days is considered a failed discharge and no second inpatient case-weight can be invoiced.
 - Following discharge from hospital, readmission into inpatient rehabilitation between 8 days and 180 days for the same injury, following an inpatient interRAI or manual inpatient profiling tool reassessment a second case-weight can be invoiced.
 - Readmission into inpatient rehabilitation greater than 181 days post discharge will require Te Whatu Ora District Providers to complete the ACC 7985 Exceptional Circumstances form. ACC needs to give prior approval before a second case-weight can be invoiced.

Where a client requires transitional care, the community profiling tool can be utilised a maximum of two times. However, NARP Community Pathway Group 4 cannot be used twice.

- Rehabilitation plans will provide the expected timeframes for the duration required for each pathway.

- When a client completes their NARP service pathway and further injury related rehabilitation needs are identified, the supplier will notify ACC using the ACC7985 Exceptional Circumstance form.
- When a client is discharged from inpatient care and is recommended to only require physiotherapy intervention provided by a private practice physiotherapist, notification to ACC is not required.
- When a client with cover for a Treatment Injury is readmitted to inpatient rehab for an injury related need, the supplier will notify ACC using the ACC7985 Exceptional Circumstances form.

Note: Treatment injury claims are outside of the contract service item code case-weights

See NARP service schedule Part A Table 4

NRPTI	Treatment Injury - Inpatient	For reimbursement of a Client who has had an accepted treatment injury. Not to be used in conjunction with the case weighted inpatient packages from Table 1. Requires prior approval from ACC.	\$1,278.55	Per day
NRPTC	Treatment Injury - Community	For reimbursement of a Client who has had an accepted treatment. Not to be used in conjunction with the community case weighted packages from Table 2. Requires prior approval from ACC	\$156.48	Per day

Service Pathway Assessment Profiling

(Refer to Part B, Clause 7.2. of Service Schedule)

The Supplier will determine the Client's service pathway by carrying out an interRAI assessment on admission to each stage of the pathway.

The purpose of the NARP interRAI assessment is to identify the most appropriate case-weight service item code and the package of care, based on the Client's needs. Following allocation to the most appropriate package of care from the interRAI assessment, the Supplier will develop a rehabilitation plan with the Client, their family/whānau and primary carer.

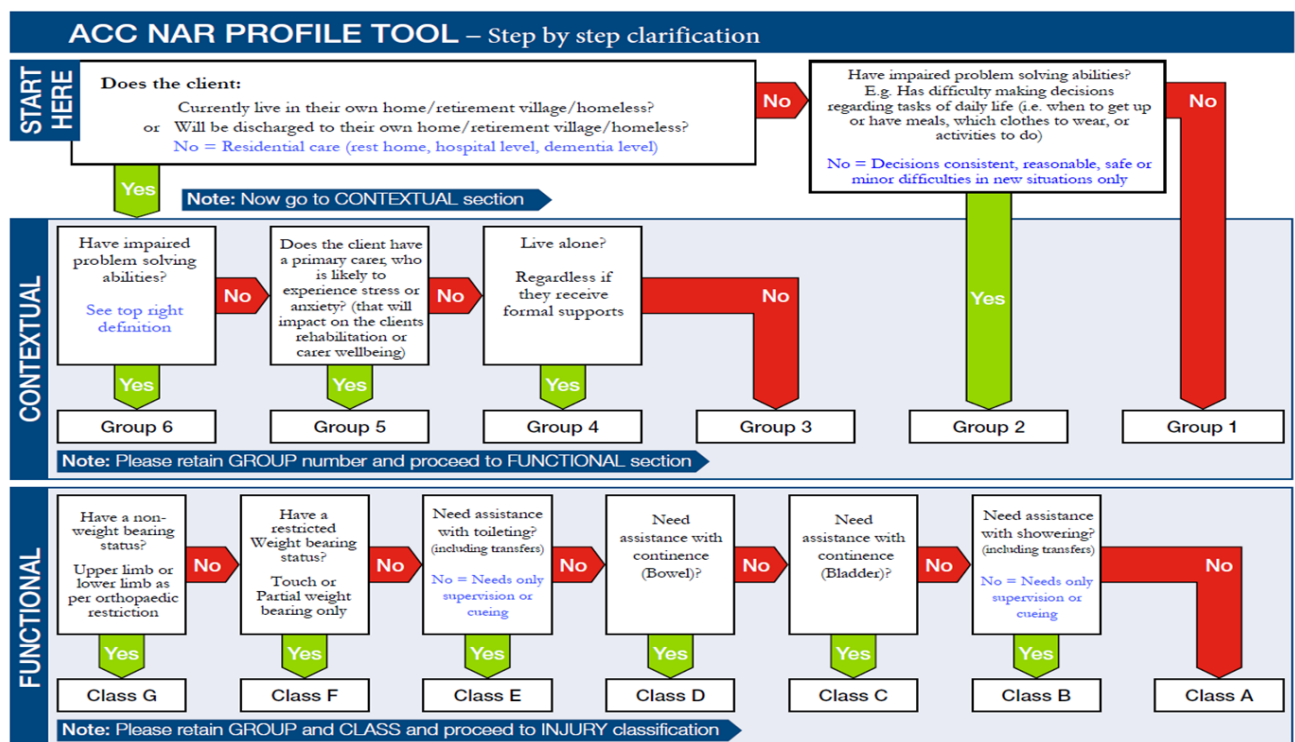
There are two manual profiling tools:

Inpatient profiling tool

The manual Inpatient profiling tool is embedded into the interRAI Acute Care Admission assessment and is no longer needed to be manually assessed, when District Providers are using interRAI.

The Inpatient Profiling tool is applied by a clinician in the inpatient setting as part of an interRAI comprehensive assessment. The tool considers the Client's living arrangement, contextual environment, level of functioning and injury. The Client is then allocated into a group and class which reflects the Client's needs. The Supplier will provide Inpatient Rehabilitation in line with the Minimum Services Requirements. (refer above page 13)

Inpatient Rehabilitation is location agnostic and can occur either in the hospital rehabilitation ward, a satellite hospital or a residential facility. All components of the inpatient rehabilitation pathway must be provided in any of the settings.



Note: Inpatient Rehabilitation profiling tool can be found on the [ACC NARP webpage](#)

Figure 6: Inpatient profiling tool - source Auckland University

Exceptions to the use of the Inpatient profiling tool

Exceptionally Complex clients **do not** require an interRAI assessment, as a case-weight service item code is not required, as they are paid on per day rate. Exceptionally Complex clients include:

- Treatment Injury - Inpatient
- Treatment Injury - Community
- Inpatient Rehabilitation - Serious burns
- Inpatient Rehabilitation – Specialised rehabilitation required for complex injuries, such as Spinal Cord injury or Severe Traumatic Brain injury, while awaiting transfer to specialised rehabilitation center.

Note: Suppliers must use the ACC7985 for Exceptional Circumstances

Community profiling tool

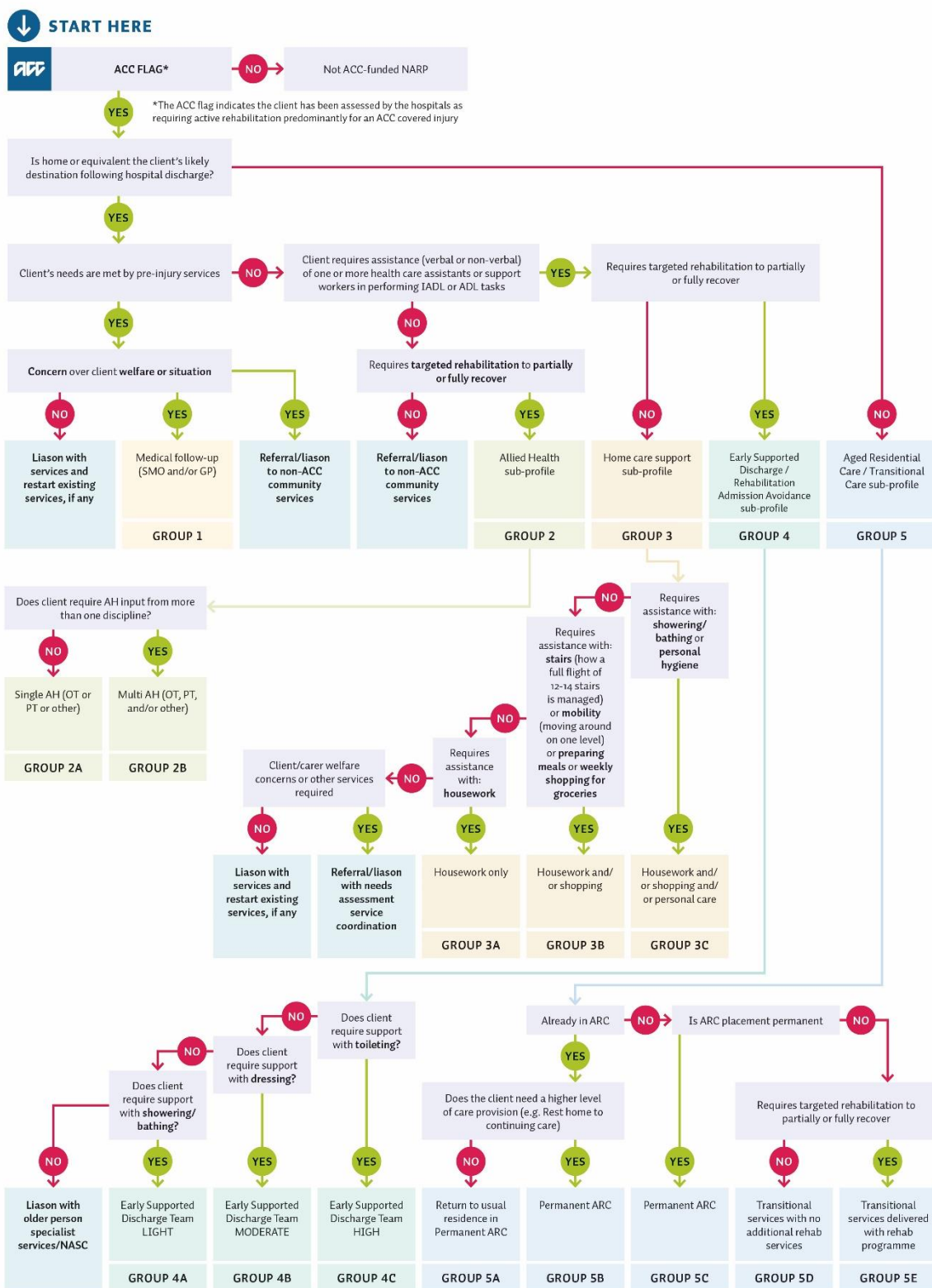
The purpose of the Community Profiling tool is to identify the most appropriate casemix grouping and thus community package of care based on the Client's needs.

The Community Profiling tool is utilised as part of a comprehensive assessment when the client is identified as being safe to move into the community setting. This can occur from an emergency department, acute ward or inpatient rehabilitation setting. The manual Community profiling tool is also embedded into the interRAI Acute Care Discharge assessment. When a client has had inpatient non-acute rehabilitation admission and the District Provider is using interRAI, this no longer needs to be manually assessed.

The tool considers the Client's living arrangement, contextual environment, level of functioning and injury. The Client is then allocated into a group and subgroup which reflects the Client's needs.

Where a client has had a NARP inpatient rehabilitation or transitional care episode, they can access all NARP community pathways.

Where a client has NOT had a NARP inpatient rehabilitation episode they can only access NARP community pathway 4 (This journey directly to community group 4 is called Rehabilitation Admission Avoidance).



Note: Community Rehabilitation profiling tool can be found on the [ACC NARP webpage](#)

Note: The ACC flag indicates the Client has been assessed by the hospitals as requiring active rehabilitation for an ACC covered injury only

Figure 7: Community profiling tool overview - source Auckland University

Service Pathways

(Refer to Part B, Clause 8 of Service Schedule)

The Client's journey through the NARP services may include one or a combination of the following Rehabilitation Pathways, depending on the Client's clinical needs for the accepted injury.

- Inpatient Rehabilitation
- Transitional Rehabilitation
- Community Rehabilitation
- Rehabilitation Admission Avoidance

Patient scenarios for Non-Acute Rehabilitation Pathway (NARP) services

NARP enables districts to move patients between services and settings to match their need. The client remains the responsibility of Te Whatu Ora until they exit all NARP services.

The following scenarios show four typical pathways for patients through the NARP service from acute admission to discharge.

Scenario one: Vijay – Inpatient and Community Rehabilitation Pathway

Vijay is an 83-year-old who sustained had a fractured neck of femur after falling off a ladder. He lives at home with his wife and is usually fully independent.

After his accident Vijay presented to ED and was admitted to the orthopaedics ward. Vijay had surgery to address his fracture.

After surgery, Vijay needed a two-person assist to transfer with a walking frame and assistance with personal care. Vijay's clinical team decided he would benefit from a period of rehabilitation and that he met eligibility criteria for NARP services. Vijay accessed inpatient rehabilitation, as well as community rehabilitation through NARP services.

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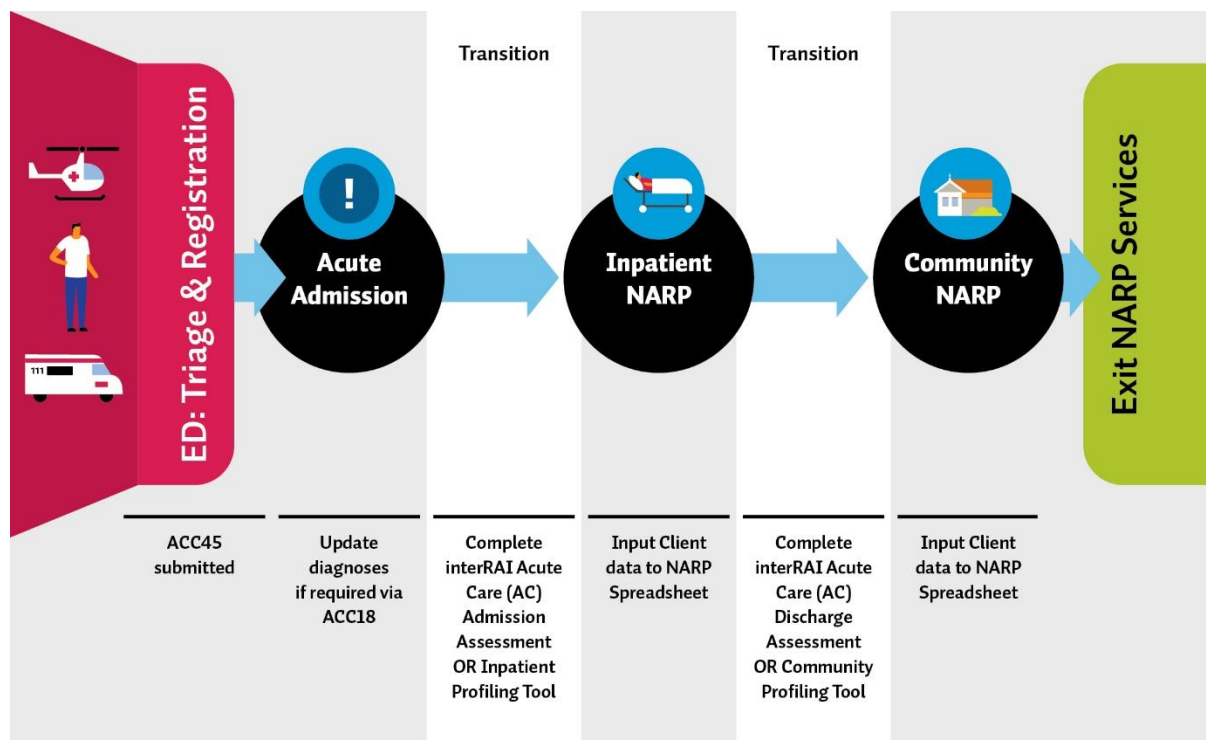


Figure 1: Scenario one -Vijay's Pathway through NARP services

Scenario two: Talita – Rehabilitation Admission Avoidance Pathway

Talita is a 75-year-old who fractured her left neck of humerus after she fell while gardening. She lives at home alone and is usually independent with all activities of daily living. After her accident, Talita presented to ED and was admitted to the orthopaedics ward. She didn't need surgery, was treated conservatively and given a sling to wear. While Talita could walk independently for short distances and needed assistance with personal care and getting out and about in the community.

Talita met eligibility criteria for NARP services. Her clinical team decided that, with the support of a multidisciplinary community rehabilitation team, Talita could safely return home and finish her rehabilitation in the community without needing inpatient care.

In this scenario, the clinical team decided to use the Rehabilitation Admission Avoidance pathway. Talita is then able to access NARP Group 4 Rehabilitation Admission Avoidance service.

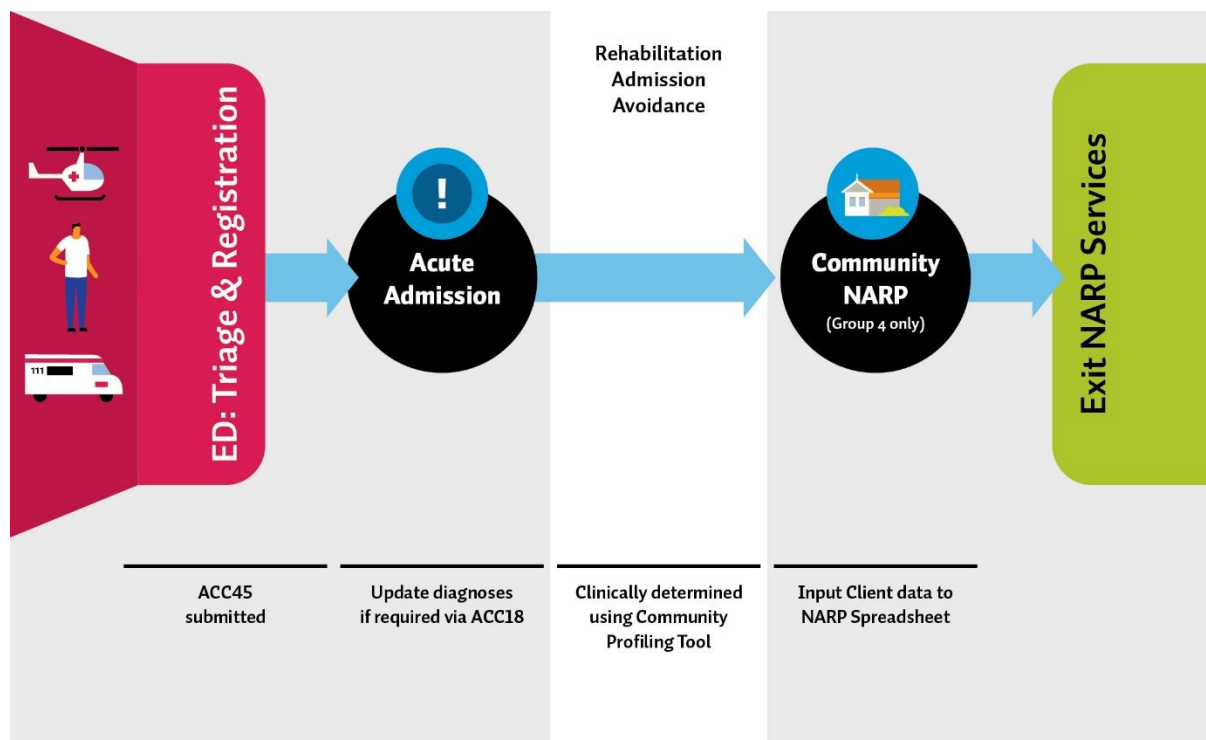


Figure 2: Rehabilitation Admission Avoidance Pathway

Scenario three: Wiremu – Transitional, Inpatient and Community Pathway

Wiremu is an 81-year-old who fractured his right neck of femur after a fall while walking on the beach. He usually lives at home with his wife and is independent with a walking stick. After his accident, Wiremu presented to ED and was admitted to the orthopaedic ward. Wiremu had surgery and was required to be non-weightbearing for six weeks. He was able to transfer with a walking frame and assistance of one and needed assistance with all personal cares.

Wiremu's clinical team decided he would benefit from some time in transitional care and used the community profiling tool to determine the case mix. He met the eligibility criteria for NARP services.

After six weeks of transitional care, the clinical team determined that Wiremu needed inpatient rehabilitation. The inpatient profiling tool was used on admission to inpatients. Following this, the clinical team then determined that Wiremu needed a period of community rehabilitation, and the community profiling tool was used for the second time in Wiremu's NARP journey.

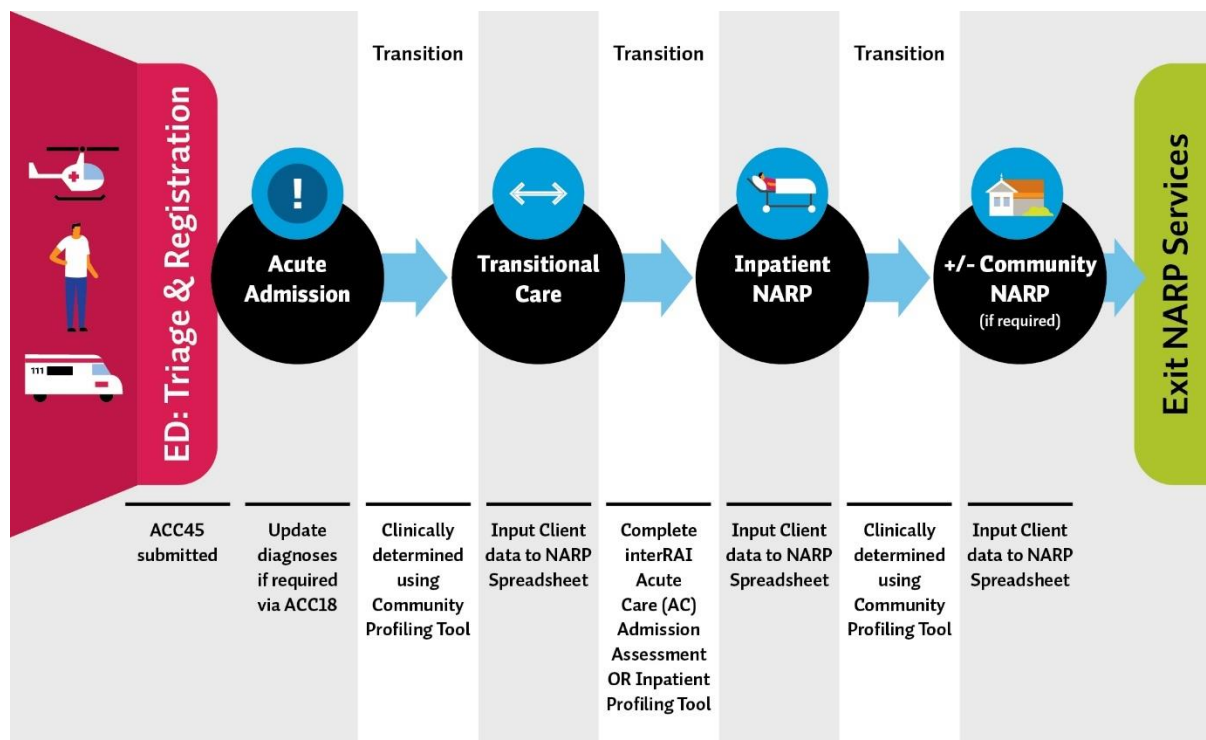


Figure 3: Transitional, Inpatient and Community Pathway

Scenario four: Agnes – Transitional and Community Pathway

Agnes is a 78-year-old who fractured her right neck of femur in a car accident. Agnes usually lives at home with her husband and walks independently without an aid. Agnes presented to ED and was admitted to the orthopaedic ward where she had surgery. Agnes was required to be non-weightbearing for six weeks. Her clinical team decided that Agnes would benefit from a period of transitional care where she could actively engage in rehabilitation.

Six weeks' later, Agnes was cleared to weight bear as tolerated. Agnes was profiled again using the community profiling tool to establish which community pathway would be used. Agnes could begin her community rehabilitation while in transitional care which enabled a supported transition back home.

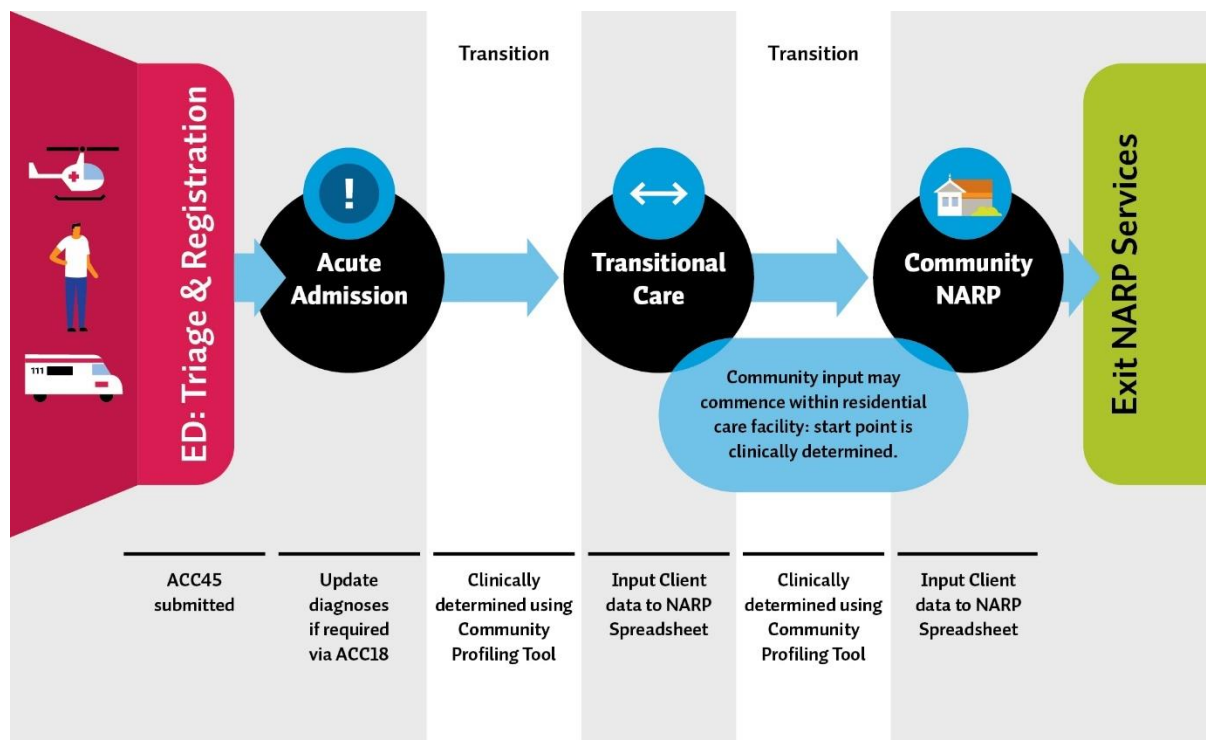
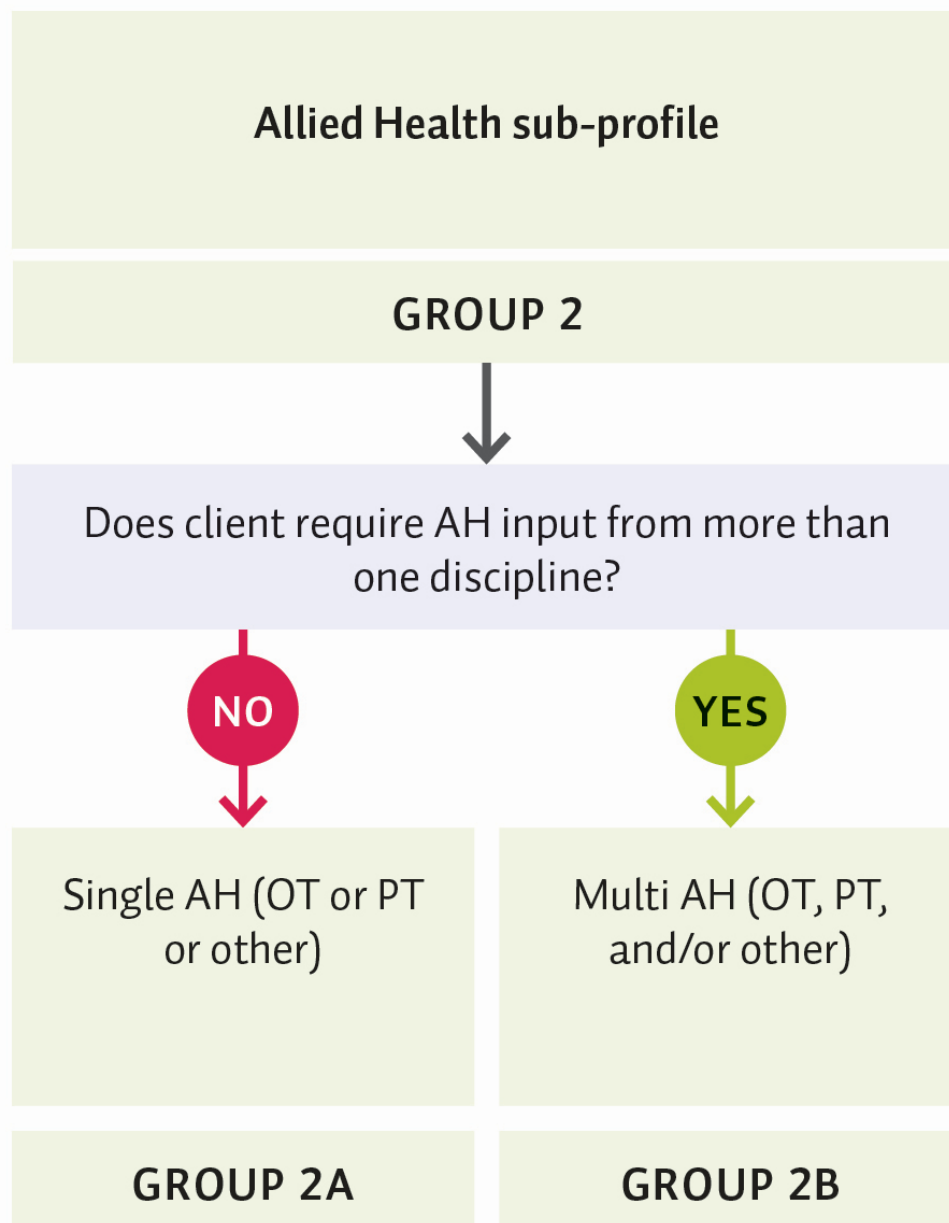


Figure 4: Transitional and Community Pathway

Community Group breakdown with scenarios

This section describes the sub-sections of the community profiling tool.



Community profiling tool group 2 (2a and 2b) – source Auckland University

Group 2 Client Scenario: (NRPC2A and NRPC2B)

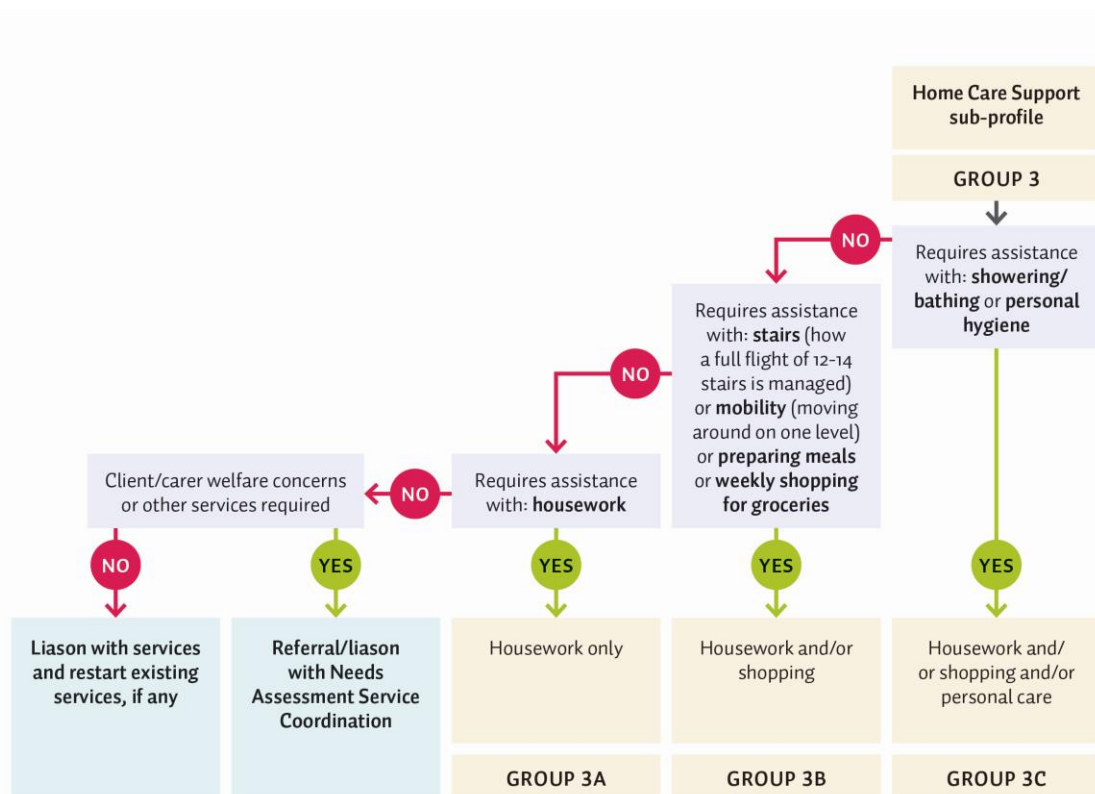
A 75 year old female had a fall at home after tripping over a mat, she sustained an ankle fracture. She is walking independently with a walking frame and is independent with personal

cares and activities of daily living. She has been cleared for discharge home from inpatient rehabilitation where she lives alone and has a steep driveway and four steps to access the house. She requires rehabilitation to progress her mobility (progressing off walking aids, accessing the home, mail box and local dairy). In this scenario the client would be classified as a Group 2a.

NOTE: Where the client has had a NARP inpatient rehabilitation or transitional care episode, the need for community allied health would be funded under NARP. Where the client has NOT had an inpatient rehabilitation or transitional care episode and there is a need for rehabilitation, ACC requires notification and will provide the appropriate rehabilitation.

An 80 year old male fell while out walking on the beach and sustained a fractured neck of femur. He is walking independently with a walking frame and has been discharged home from inpatient rehabilitation. He is able to complete personal cares and activities of daily living with adaptive equipment. He requires rehabilitation to progress off walking aids and return to completing personal cares and activities of daily living without equipment. In this scenario the client would be classified as a Group 2b

NOTE: Where the client has had a NARP inpatient rehabilitation or transitional care episode, the need for community allied health would be funded under NARP. Where the client has NOT had an inpatient rehabilitation or transitional care episode and there is a need for rehabilitation, ACC requires notification and will provide the appropriate rehabilitation.



Community profiling tool group 3 (3a, 3b and 3c) – source Auckland University

Group 3 Client Scenarios: (NRPC3A, NRPC3B and NRPC3C)

An 82 year old female sustained a significant open wound to the hand after a fall in the garden. On discharge from inpatient rehabilitation she is able to walk independently and complete all personal cares independently. She already has established support for shopping to address non injury needs however as a result of the wound requires assistance with household tasks such as making the bed, doing laundry and doing the dishes. It is expected that as the wound heals the client will return to full independence without the need for any specific rehabilitation. In this scenario, the client would be classified as a Group 3a.

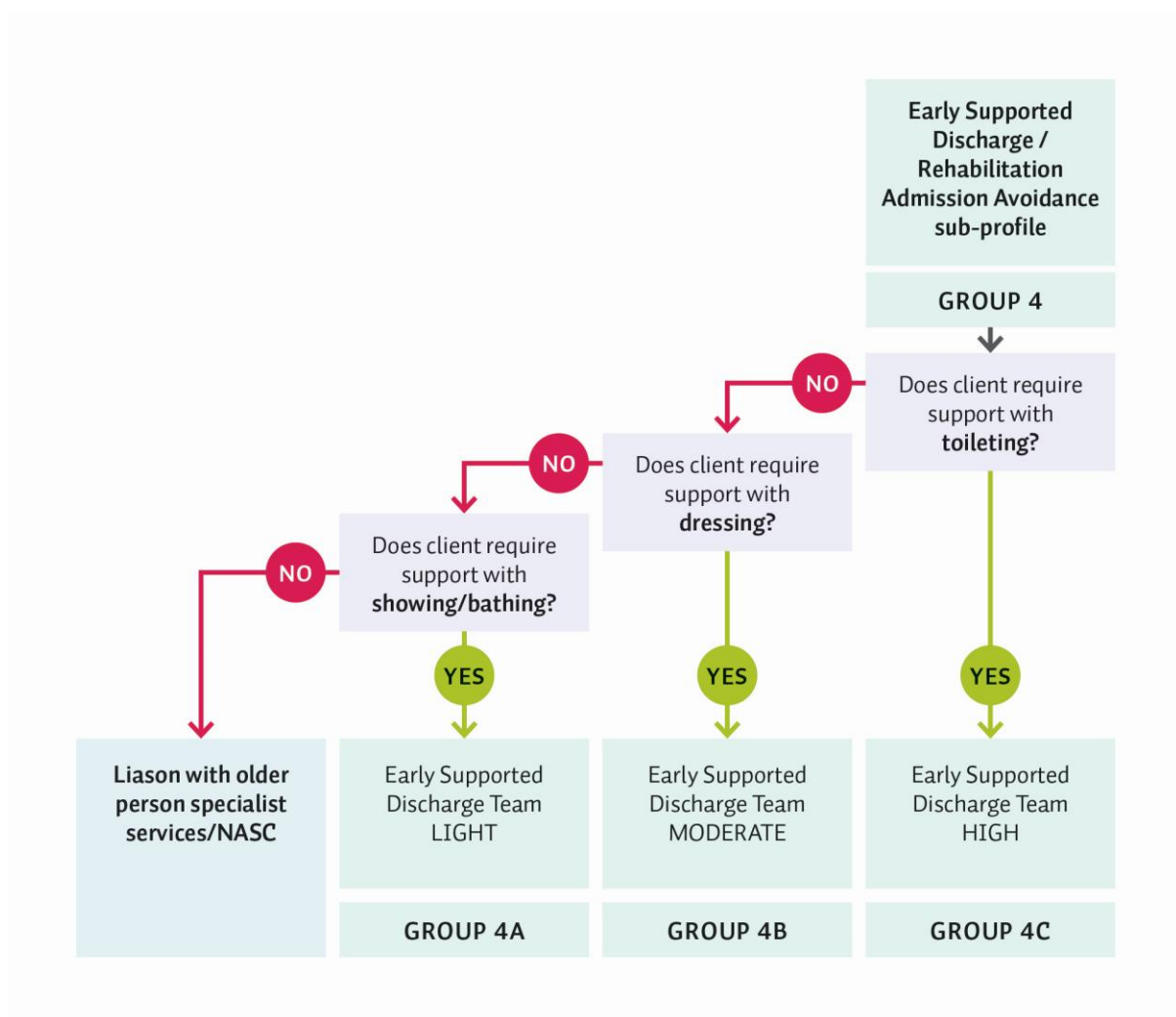
NOTE: Where the client has had a NARP inpatient rehabilitation or transitional care episode, the need for community home supports would be funded under NARP. Where the client has NOT had an inpatient rehabilitation or transitional care episode the need for community home supports would be funded through ACC's home and community support services.

A 78 year old male slipped on a ladder and sustained a rib fracture. On discharge from inpatient rehabilitation he is able to walk independently unaided however is unable to lift heavy items and lives alone. He is able to complete personal cares independently but needs assistance with transporting shopping. It is expected that as the rib fracture heals the client will return to full independence without the need for any specific rehabilitation. In this scenario, the client would be classified as a Group 3b.

NOTE: Where the client has had a NARP inpatient rehabilitation or transitional care episode, the need for community home supports would be funded under NARP. Where the client has NOT had an inpatient rehabilitation or transitional care episode the need for community home supports would be funded through ACC's home and community support services.

A 65 year old female was involved in a road traffic accident and sustained a number of significant lacerations affecting the upper and lower limbs. On discharge from inpatient rehabilitation she is able to walk very short distances indoors independently however requires assistance with getting in and out of the shower and with washing. She also requires assistance with doing laundry, cleaning the home and accessing the local shops for groceries. The client lives alone. It is expected that as the lacerations heal, the client will return to her fully independent baseline without the need for specific rehabilitation. In this scenario, the client would be classified as a Group 3c.

NOTE: Where the client has had a NARP inpatient rehabilitation or transitional care episode, the need for community home supports would be funded under NARP. Where the client has NOT had an inpatient rehabilitation or transitional care episode the need for community home supports would be funded through ACC's home and community support services.



Community profiling tool group 4 (4a, 4b and 4c) – source Auckland University

Group 4 Client Scenarios: (NRPC4A, NRPC4B and NRPC4C)

A 73 year old male sustained a tibial plateau fracture after a cycling accident. He is able to mobilise independently using elbow crutches and has been deemed safe for discharge home. He requires assistance with transferring in and out of the shower however is able to dress himself and toilet independently. The client will require rehabilitation input to progress off elbow crutches and increase lower limb strength and range of motion however would be expected to complete a home exercise programme independently. In this scenario, the client would be classified as a Group 4a as the input required from the early supported discharge team would be considered light.

A 72 year old female sustained multiple pubic rami fractures after a fall while walking her dog. She is able to walk short distances with a walking frame independently and has been deemed safe for discharge home. She is able to toilet independently however requires assistance with dressing and showering. She also requires rehabilitation input to progress off the walking frame and return to independence with personal care tasks. In this scenario the client would be classified as a Group 4b as the input required from the early supported discharge team would be considered medium.

A 60 year old male sustained bilateral wrist fractures as well as an avulsion fracture of the left ankle managed in moonboot a result of a motorbike accident. He is able to mobilise

independently short distances and had been deemed safe to return home with supports in place. He requires assistance with toileting, dressing and showering as well as rehabilitation. Given nature of the client's injuries and requirement for support and rehabilitation, in this scenario the client would be classified as a Group 4c.

Note: The Light / Moderate / High sub-groups within Group 4 relate to the level of clinical input required, the complexity of the injury and how long the Client may take to recover.

Rehabilitation Admission Avoidance (AA)

Use of the Rehabilitation Admission Avoidance pathway is based on clinical decision making.

The Rehabilitation Admission Avoidance pathway is for Clients who, with timely input from a multidisciplinary community rehabilitation team, (Group 4 only) can avoid inpatient rehabilitation and can go straight to receiving community rehabilitation.

Table 3: Rehabilitation Admission Avoidance Pathways Service requirements summary

Rehabilitation Admission Avoidance Profiling Group	Sub-profile	Descriptor	Expected Timeframes
Group 4 AA	Multi-Disciplinary Team - Early Supported Discharge/ Rehab Admission Avoidance	Where the client requires Early Supported Discharge (ESD) or Rehab Admission Avoidance Group 4 a: ESD/RAA Team – light <i>(client requires targeted rehabilitation input and can toilet and dress themselves but requires support with showering/bathing)</i> Group 4 b: ESD/RAA Team – moderate <i>(client requires targeted rehabilitation input and can toilet themselves but requires support with showering/bathing and dressing)</i> Group 4 c: ESD/RAA Team – high <i>(client requires targeted rehabilitation input and requires support with toileting, showering/bathing and dressing)</i>	Maximum 12 weeks from start date

Linkages

(Refer to Part B, Clause 15 of Service Schedule)

The Supplier must ensure that linkages are maintained with the following Services:

- General practice medical services
- Drug and alcohol services
- Mental health services
- Education sector
- Māori health sector
- Other appropriate ethnic and cultural groups
- Government departments and agencies such as Police, Work and Income, Ministry of Social Development, Housing NZ, Ministry of Health, Ministry of Justice
- Disability consumer groups such as the Brain Injury Association of New Zealand (BIANZ)
- Community based day programmes, independent of those that may be operated by the Supplier.

Reporting Requirements

(Refer to Part B, Clause 17 of Service Schedule)

Business and Administration Rules

This section includes information on:

- Collecting and entering NARP data
- Rules on collecting data
- Forms required

Collecting and Entering NARP Data

The Supplier is responsible for collecting the data from profiling and entering it into a master spreadsheet that is provided by ACC or alternative data portals used by Districts.

Data points required include:

Provider ID

NHI

Client First Name

Client Surname

ACC Claim Number

Service (Inpatient/Community/Transitional)

Code/Group Class

Major Injury Type

- Burns_and_Complications_of_procedures

- Fracture_lumbar_spine_pelvis_shoulder_and_upper_arm
- Other_fractures
- Head_Neck_Thorax_and_Abdomen_Non_Fracture
- Fracture_of_Femur
- Inured_Upper_and_Lower_Limbs

ICD10 Code

- Service Start Date (Pathway start date)
- Service Discharge Date
- DHB Invoice Number
- Inpatient EDD (Calculated. This is built into both the master spreadsheet and interRAI)

Also to be included if not using the Master Spreadsheet (the Master Spreadsheet auto populates this field).

- Cost (case weight)

This data will be used to create an admission and discharge CSV file that hospitals will be required to submit to ACC either through the integrated data exchange (IDE) or SendInvoice.

Whether using CSV or another billing method, the same format is required. The comments field needs to be set out in this format:

Group Class; Major Injury Type; ICD10 Code; Discharge Date; EDD

For example:

4A;Fracture_lumbar_spine_pelvis_shoulder_and_upper_arm;S32 Fracture of lumbar spine and pelvis;25/07/2023;9/07/2023

The Master Spreadsheet and Data Exchange Guide can be found [here](#).

A report can be pulled from interRAI to capture all new assessments. This does not need to be done one claim at a time.

The Supplier is responsible for their internal processes to collate the required data into the master spreadsheet. The rules on collecting data are covered in the next section.

The information from profiling is entered into a master spreadsheet. This spreadsheet:

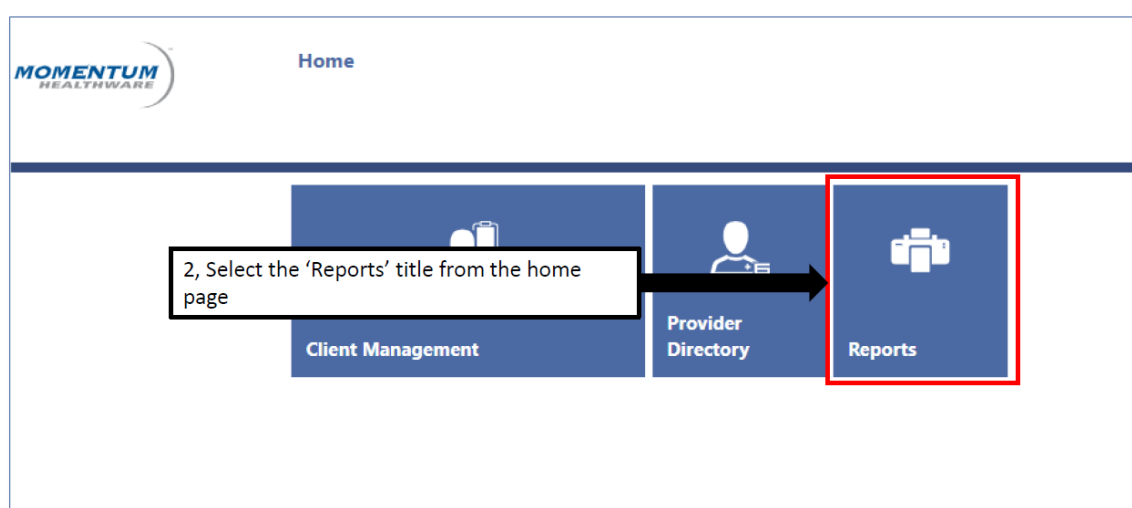
- collects service information
- captures the case-weight grouping
- collates time stamps to support service monitoring and outcomes reporting

Entering the inpatient profiling information into the master spreadsheet signals the expected date of discharge which supports care and discharge planning.

ACC will monitor the quality and timeliness of data submission and from suppliers. When there are issues, ACC will work collaboratively with Suppliers to support improvements ACC will make prompt payment on receipt of the monthly master spreadsheet.

How to access Non Acute Rehabilitation Inpatient Detail Case-mix Report

- Log into interRAI/Momentum
- On the Home screen, click on 'Reports' icon



- Select 'Non Acute Rehabilitation (NAR) Inpatient Detail Case-mix Report' OR 'Non Acute Rehabilitation Community Case-mix Report'

Select Report			
	Report Title	Report Category	Description
	Infections Tracking History	Integrated Report	Client Infection Tracking History
	Infections Tracking Monthly Summary	Integrated Report	Displays the number of infections for each infection type by month. 12 months.
	InterRAI CAP Trending By Client	Integrated Report	InterRAI CAP Trend Analysis By Resident
	InterRAI Outcome Analysis for LTC/HC - By Client	Integrated Report	InterRAI Outcome Analysis for LTC/HC - By Client
	InterRAI Vulnerable Persons at Risk Scale for Home Care Report	New Zealand	InterRAI Vulnerable Persons at Risk Scale Report
	LTCF Active User List Report	New Zealand	LTCF Active User List Report
	MDS Assessment Tracking Report - By Due Date	Integrated Report	MDS Assessment Tracking By Due Date
	MDS Assessment Tracking Report - By Last MDS / Assessment	Integrated Report	MDS Assessment Tracking By Last Assessment
	MDS Assessment Tracking Report - By Name	Integrated Report	MDS Assessment Tracking By Name
	Non Acute Rehabilitation (NAR) Community Case-mix Report	Integrated Report	InterRAI Acute Care-Discharge and the calculated case
	Non Acute Rehabilitation (NAR) Inpatient Detail Case-mix Report	Integrated Report	3, Select 'Non Acute Rehabilitation (NAR) Inpatient Detail Case-mix Report' and the calculated case using the custom NZ alg
	NZ Client Case Mix Detail Report	New Zealand	InterRAI Client Case Mix Detail
	Referral Service Stream	Integrated Report	Referral Service Stream
	Resident Listing - Assessments 30-day report	New Zealand	All assessments completed in last 30 days, plus assessments due and overdue in the next 30 days.
	Resident Listing - Assessments by Reason for Assessment	New Zealand	
	Resident Listing w/ Advanced Directives and Vital Signs	New Zealand	Resident Listing report with diagnoses, primary contact person details, advanced directives, vital signs and weight/BMI.
	Resident Listing with Ethnicity	New Zealand	List of residents and their identified ethnicities
	RUG Classification Summary	Integrated Report	RUG Classification Summary
	RUG Report	Integrated Report	RUG Report
	Weight and BMI - By Client	Integrated Report	Resident Listing Weight BMI

- Select View Report
- Select the format for the report, the preferred format is "CSV"
- Enter the filters for the report

The screenshot shows a web form titled "Report Criteria - Non Acute Rehabilitation (NAR) Inpatient Detail Case-mix Report". It includes fields for "Last Name", "First Name", "Primary ID", "Start Date", "End Date", "Record Status" (set to "Active"), and "Organization". A "View Report" button is highlighted with a red box and labeled "6, Click on 'View Report'". A dropdown menu for report format is open, showing options like "PDF", "Excel 97 or later", "Excel 2007 or later", "CSV", "Word 97 or later", "Word 2007 or later", and "Powerpoint 2007 or later". The "CSV" option is highlighted in yellow and labeled "5, Select the format for the report, the preferred format is 'CSV'". Arrows point to the "Start Date" and "End Date" fields, labeled "4, Enter the filters for the report".

Rules on collecting data

The table provides the rules and explanation for data collection for inpatient care.

Table 10 Inpatient rules

Topic	Rule	Comment
Adding an admission	The hospital will assess and profile the Client using the appropriate profiling tool. The profiling information will be collated into the admission spreadsheet when the Client enters the service.	Admission date to NARP (Start active rehab) is the day the Client first started participating in rehabilitation which may or may not be the day the Client came into the unit.
Interruptions in care	Interruptions in care of three days or less are ignored. An interruption of care of greater than three days requires notification to ACC and the client is to be transferred to hospital funding for the duration of the interruption.	These will be absorbed into the inpatient length of stay of the Client. The hospital will notify ACC via email claimsdocs@acc.co.nz
Readmission 0 – 7 days after the 1st discharge	Failed discharge: Any injury related readmission against the same claim within 7 days of discharge from inpatient NARP is seen as a failed discharge. When the Client is discharged again the existing line in spreadsheet is updated with new discharge date. There will be no further case-weight assigned.	Each admission has a corresponding discharge.
Readmission Between 8 – 180 days after 1st discharge	Any injury related readmission against the same claim between 8-180 days after 1st discharge from inpatient NARP services the client will be readmitted under the same injury and will be profiled again using the inpatient profile tool for access to a secondary case-weight code payment and the readmission tab on the master spreadsheet will need to be updated.	Note that only the 1st admission profile is included in the admission and discharge invoice. Any subsequent admissions will require the discharge date for 2 nd inpatient invoicing

Readmission 181 days after 1st discharge	Any injury related readmission against same claim number greater than 181 days since 1st discharge is seen as an exception and needs to be pre-approved by ACC.	Please use the 'NARP Exceptional Circumstances form ACC7985' to apply for this exception
Inpatient discharge date	The date of discharge from NARP inpatient rehabilitation is captured to allow invoicing to occur.	Both admission and discharge dates are required to send invoice. This information will allow collection of data on inpatient length of stay

Forms required

(Refer to Part B, Clause 11 of Service Schedule)

Table 3: NARP Service forms

Form number and name	Purpose	Explanation	Example scenario
ACC7985 Exceptional Circumstances Notifications	This form informs ACC that there is an exceptional circumstance which requires ACC approval	When a hospital identifies that one of the exceptional circumstances outlined in Table 7 has occurred they will complete the form and send to ACC	See Table 7 for further detail on exceptional circumstances
ACC705 Referral for support services on discharge	This form outlines the support the Client requires for injury related needs on discharge from supplier's care. This form should be used for patients when the NARP services criteria are not met and specific ACC services are required; i.e.: equipment, notification that the client requires residential support.	The ACC705 is used to notify ACC of client needs or circumstances not addressed in the ACC7985 Exceptional Circumstances form. Suppliers without Community Rehabilitation pathways in place will use this form to notify ACC of any discharge injury needs.	The client is discharging from inpatient or community rehabilitation pathway and requires home modifications or complex equipment: i.e. Wheelchair, permanent ramp

NOTE:

ACC [NARP webpage](#) is where you find the forms in table 11. These will be updated as needed so it is important to ensure the correct version is being used. Using outdated forms increases the risk of delays in process requests.